

1. Overview

UpDoc appreciates the opportunity to respond to the HHS Office of the Deputy Secretary and ASTP Request for Information on accelerating the adoption and use of artificial intelligence in clinical care.

UpDoc is a software medical device developer that produces physician-supervised clinical AI workflow solutions designed to extend care capacity while preserving physician accountability. Our platform enables AI-facilitated clinical interventions that operate exclusively within preauthorized, physician-defined care pathways, and under the direction of a prescribing physician. All actions are digitally documented and auditable. Consequently, the suggestions that are proposed in this RFI Response only apply to AI systems that facilitate interventions under the oversight and prescription of a physician.

We believe the principal barriers to responsible AI adoption are misalignment between existing reimbursement structures and workflow-integrated AI healthcare delivery. Current time-based fee-for-service billing models do not include productivity-enhancing tools. While CMS has accommodated coverage and payment for certain Software as a Medical Device (SaMD) medical equipment under Medicare Part B physician services, it has only done so in a few instances and by forced cross-walks that are largely inapplicable. The lack of predictable coverage and payment for productivity-enhancing medical device software may fragment care delivery and weaken physician adoption.

We respectfully offer three guiding principles for federal policy consideration:

1. Payments should increasingly reimburse physicians for both care outputs and validated workflows, not only for software licenses or time.
2. AI-enhanced clinical care must remain physician-anchored, with defined guardrails that preserve accountability and safety.
3. Digitized workflows enable both scalability and oversight, providing auditability, fraud mitigation, and a pathway toward outcome-based reimbursement models.

Our responses below address the specific questions posed in the RFI.

Please note: This response is NOT intended to provide feedback for facilitation of AI systems that deliver care independently of physician oversight or prescription authority that may involve other legal state authorities, governance structures, and other payment regimes.

2. Responses to Specific Questions

Response to Question 1: What barriers impede adoption of AI in clinical care?

Direct Answer:

The primary barriers to adoption are structural reimbursement misalignment within existing CMS payment methodologies, such as time-based billing constructs that do not account for improvements in efficiency. There presently does not exist accountability frameworks for AI-enabled clinical actions.

Explanation:

Under the Physician Fee Schedule methodology, medical device software expenses are generally treated as indirect, non-allocable practice expenses rather than direct medical equipment inputs. This categorization creates structural reimbursement barriers for workflow-integrated AI medical device software technologies which are not generic off-the-shelf IT.

When workflow-integrated AI functions as core clinical infrastructure, classification as indirect practice expense fails to reflect its role in delivering physician-directed care.

In addition, many asynchronous care services and codes (e.g., Digital E/M, Care Management Services such as CCM, RPM, etc.) require documentation of human time spent supervising care. When AI systems increase productivity and reduce required clinician time, physicians may paradoxically experience revenue reduction, creating a financial disincentive to adoption.

Organizations face uncertainty as AI-enabled workflows are operationalized. Clear federal guidance reinforcing coverage and payment of software as a medical device-enabled workflows would materially reduce uncertainty and accelerate responsible adoption.

Response to Question 2: What payment policy changes would accelerate adoption?

Direct Answer:

CMS should establish clear, non-time-based reimbursement pathways for asynchronous care, much like it has begun to do by creating Advanced Primary Care Management (APCM) services. CMS created APCM service codes under the existing FFS methodology to support primary care providers in delivering comprehensive, continuous, and coordinated care while omitting time-based requirements to facilitate a shift towards value-based care. Similarly, CMS should explore creating additional services (i.e. HCPCS codes) to recognize and reimburse

physician-supervised, AI-enhanced clinical actions that reflect Medical Decision Making (MDM) complexity. In parallel, CMS should broaden and strengthen reimbursement pathways for asynchronous care to align payment with the evolving clinical realities of AI-enabled care as it scales in impact and associated liability exposure beyond real-time encounters.

Explanation:

Near-term reforms could include:

- Explicit recognition that AI-facilitated clinical interventions under physician supervision contribute to MDM complexity.
- Creation of new services or HCPCS G-codes for asynchronous E/M services that leverage provider AI oversight that includes a new form of MDM.
- Clear CMS guidance that validated workflow substitution by AI may serve as a lower-cost alternative to face-to-face encounters where clinically appropriate.

These clarifications could be implemented within existing statutory authority and CPT infrastructure without requiring new legislation, enabling productivity-enhancing AI to increase care capacity without penalizing physicians economically.

Response to Question 3: Legal and governance considerations**Direct Answer:**

AI-enabled clinical care should operate within human physician-anchored accountability frameworks that preserve prescribing authority, enforce deterministic care boundaries, and ensure full auditability of interventions.

As AI capabilities evolve, governance frameworks should scale proportionately, ensuring that systems operating with higher degrees of autonomy are accompanied by commensurate human-in-the-loop safeguards, transparency, and accountability.

Explanation:

AI systems delivering clinical interventions under physician oversight and payment structures should:

- Operate exclusively within preauthorized, physician-defined care pathways.
- Block actions outside approved clinical parameters.
- Maintain comprehensive digital logs of all patient interactions and interventions.

- Reinforce that ultimate clinical responsibility remains with the supervising physician, and that AI systems should not independently initiate or modify treatment plans outside physician-defined parameters.

The above applies only to AI systems that facilitate interventions under the oversight and prescription of a physician. For these systems, payment models that operate independently of physician-led workflows may create fragmentation and unclear accountability pathways, and federal guidance should prioritize integrated, physician-led models. **The above does not apply to AI systems that deliver care independently of physician oversight or prescription authority that may involve other legal state authorities, governance frameworks and payment regimes.**

Response to Question 4: Evaluation Methods

Direct Answer:

AI-enabled clinical care should be evaluated at the clinical workflow level, with emphasis on real-world evidence, pathway adherence, substitution effects, safety monitoring, and auditability rather than solely on isolated algorithmic performance metrics.

Explanation:

Evaluation frameworks should assess:

- Clinical outcomes achieved through AI-facilitated workflow substitution.
- Adherence to defined, physician-approved care pathways.
- Safety, utilization, and outcome metrics in real-world settings.
- Reduction in higher-cost synchronous encounters where clinically appropriate.
- Embedded digital audit capabilities supporting post-market monitoring.

Evaluation should also assess whether AI-enabled workflows substitute for higher-cost encounters without compromising clinical quality and should recognize that AI systems deployed within bounded clinical workflows differ materially from standalone algorithmic tools.

Response to Question 5: Accreditation or Certification Pathways

Direct Answer:

Accreditation frameworks should distinguish between AI systems that facilitate interventions under the oversight and prescription of a physician, and AI systems that deliver care independently of physician oversight or prescription authority.

For AI systems that facilitate interventions under the oversight and prescription of a physician, physician-supervised AI workflows should be evaluated for transparency, guardrails, and accountability in addition to algorithmic validation. There should be proportional oversight standards to systems operating with higher degrees of independent clinical authority.

Explanation:

Accreditation should emphasize system-level safeguards, not only algorithm validation in isolation.

Certification standards may include:

- Clear physician oversight and accountability structures.
- Configuration controls that prevent out-of-pathway actions.
- Inspectable audit logs documenting all AI-facilitated interventions.
- Workflow-level transparency regarding how clinical actions are initiated and validated.

Such standards would reinforce trust while allowing innovation to scale responsibly.

Response to Question 6: Where AI Has Succeeded or Fallen Short**Direct Answer:**

AI has shown success in structured, protocol-driven care environments but has faced challenges in fragmented spaces that operate outside physician-led care coordination.

Explanation:

Earlier generations of standalone Software as a Medical Device (SaMD) tools demonstrated strong algorithmic performance but often struggled to achieve durable adoption when not embedded within physician-led workflows.

Success is most durable when AI augments physician capacity rather than bypassing established care relationships.

AI performs well in medication titration, chronic disease management, and other longitudinal, protocol-driven care management settings where clinical boundaries are clearly defined by physicians who give authorization for AI to join care teams under their pre-set conditions. Adoption is more limited when reimbursement structures separate AI-enabled services from established care teams or create economic disincentives for physician participation. Such

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reimbursement structures can result in physician-AI discoordination, frequent AI-triggered safety pauses, or higher risks of safety events.

Response to Question 7: Organizational Adoption Barriers

Direct Answer:

Organizational barriers include revenue cannibalization risk under time-based billing, uncertainty regarding liability allocation and compliance expectations, integration complexity within existing care infrastructure, and change management challenges within clinical teams.

Explanation:

Health systems are reluctant to adopt technologies that may reduce billable time without corresponding payment recognition. Clear MDM-based reimbursement pathways would reduce financial uncertainty.

In addition, ambiguity regarding physician accountability and compliance expectations can slow internal approval processes. Clear federal coding / reimbursement action and guidance reinforcing physician-supervised AI models would mitigate this hesitation.

Finally, AI adoption requires integration into existing EHR systems, care team workflows, and operational governance structures. Interoperability alignment and implementation clarity are essential to reduce integration burden.

Published analyses have demonstrated meaningful reductions in synchronous visit utilization when AI-enabled workflows are properly integrated into physician-supervised care models.

Response to Question 8: Interoperability

Direct Answer:

AI-enabled workflows require structured, interoperable data aligned with ONC-adopted interoperability standards to ensure safe, scalable integration into clinical systems.

Explanation:

Standardized data exchange is essential for ensuring AI interventions are context-aware, clinically appropriate, and safely integrated into existing care workflows.

Data liquidity enables substitution of encounters, real-time documentation, and quality monitoring. Alignment with federal interoperability standards reduces integration friction and supports consistent implementation across health systems.

Interoperability policy should prioritize bidirectional data exchange that supports both clinical action and transparent documentation of AI-facilitated care.

Response to Question 9: Patient and Caregiver Concerns

Direct Answer:

Patient trust depends on clear disclosure of AI involvement. For AI systems that facilitate interventions under the oversight and prescription of a physician, there should be transparent physician oversight, clinically bounded pathways that prevent overuse, and assurance that AI tools operate within defined safety constraints.

Explanation:

For AI systems that facilitate interventions under the oversight and prescription of a physician, transparent communication regarding the physician's continued oversight of AI is central to maintaining patient trust.

For these systems, digitized workflows enhance safety monitoring and preserve accountability by ensuring all AI-facilitated interventions are documented and reviewable.

Clinically bounded, defined guardrails mitigate concerns about inappropriate utilization or overreach.

Clear policies and regulatory structures detailing physician supervision vs independent autonomy can reassure patients that AI tools are being regulated and used responsibly.

Response to Question 10: Research Priorities

Direct Answer:

HHS should prioritize research evaluating AI-enabled clinical workflow substitution models, measurable cost and utilization impacts from asynchronous care, and transition pathways toward outcome-based reimbursement frameworks.

Explanation:

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Research should measure both cost substitution effects and patient outcome metrics under controlled guardrails.

Pilot programs, including those conducted through the CMS Innovation Center, could evaluate whether physician-supervised AI workflows safely substitute for higher-cost synchronous encounters while maintaining or improving quality.

Such research should assess safety, utilization, access expansion, and longitudinal outcomes to ensure productivity gains translate into clinical value rather than excess utilization.

Evidence generated through structured pilots would inform scalable payment policy reform while maintaining accountability and patient trust.

3. Conclusion

UpDoc appreciates HHS’s leadership in examining how AI can responsibly expand clinical capacity. Clear alignment between reimbursement, governance, interoperability, and physician accountability will be essential to accelerating safe and trusted adoption of physician-supervised AI in clinical care.